

Hospital Discharge Summary

Dr. Elena Bianchi

April 29, 2025

Contents

PATIENT INFORMATION	2
ADMISSION DIAGNOSES	2
DISCHARGE DIAGNOSES	2
HOSPITAL COURSE	3
Presenting Problem	3
History of Present Illness	3
Physical Examination on Admission	3
Laboratory Findings on Admission	3
Diagnostic Studies	4
Hospital Course by System	4
Cardiovascular	4
Pulmonary	4
Renal	4
Endocrine	5
Hematologic	5
Nutrition/GI	5
Infectious Disease	5
Complications During Admission	5
Procedures	5
Consultations	5
DISCHARGE CONDITION	5
DISCHARGE LABORATORY VALUES (4/29/2025)	6
DISCHARGE MEDICATIONS	6

ALLERGIES	6
DISCHARGE INSTRUCTIONS	6
Activity	6
Diet	7
Daily Weights	7
Wound Care	7
Monitoring	7
Special Instructions	7
FOLLOW-UP APPOINTMENTS	7
PREVENTIVE CARE NEEDS	8
DISCHARGE PLANNING	8

PATIENT INFORMATION

Name: Maria Rossi
MRN: 98765432
DOB: 05/12/1952 (72 y/o)
Sex: Female
Admission Date: 04/15/2025
Discharge Date: 04/29/2025
Length of Stay: 14 days
Attending Physician: Dr. Elena Bianchi
Primary Care Provider: Dr. Marco Verdi

ADMISSION DIAGNOSES

1. Acute Decompensated Heart Failure (ADHF)
2. Non-ST-elevation Myocardial Infarction (NSTEMI)
3. Chronic Kidney Disease Stage 3b
4. Type 2 Diabetes Mellitus
5. Hypertension
6. Peripheral Vascular Disease
7. Chronic Obstructive Pulmonary Disease (COPD), GOLD Stage 2
8. History of Paroxysmal Atrial Fibrillation
9. Chronic Anemia

DISCHARGE DIAGNOSES

1. Acute Decompensated Heart Failure with preserved Ejection Fraction (HFpEF), improved
2. Non-ST-elevation Myocardial Infarction (NSTEMI), status post coronary angiography with drug-eluting stent (DES) placement to proximal LAD

3. Paroxysmal Atrial Fibrillation with Rapid Ventricular Response (AFib with RVR), rate-controlled at discharge
4. Acute Kidney Injury (AKI) on Chronic Kidney Disease (CKD) Stage 3b, improved
5. Health care-associated pneumonia (HCAP), resolved
6. Type 2 Diabetes Mellitus, poorly controlled
7. Hypertension
8. Peripheral Vascular Disease
9. COPD, GOLD Stage 2, stable
10. Iron-deficiency Anemia

HOSPITAL COURSE

Presenting Problem

72-year-old female with extensive cardiovascular history presented to the Emergency Department with 3 days of progressive dyspnea, orthopnea, and bilateral lower extremity edema. Patient reported worsening shortness of breath with minimal exertion and inability to perform ADLs without significant dyspnea. Also noted 2-pillow orthopnea, paroxysmal nocturnal dyspnea, and a 3 kg weight gain over the preceding week.

History of Present Illness

Patient has a known history of CHF (EF 54% on previous echocardiogram 6 months ago) and multiple cardiac risk factors including DM2, HTN, and PAF. One week prior to admission, she self-discontinued her furosemide due to “frequent urination” and subsequently noted progressive fluid retention and respiratory symptoms. On the day of admission, she developed retrosternal chest discomfort rated 5/10 in severity, non-radiating, associated with diaphoresis and increased dyspnea, prompting ED evaluation.

Physical Examination on Admission

- **Vital Signs:** BP 178/92 mmHg, HR 112/min irregular, RR 24/min, Temp 37.1°C, SpO2 88% on room air, improved to 94% on 2L NC
- **General:** Elderly female in moderate respiratory distress, sitting upright to breathe
- **HEENT:** Jugular venous distension to 12 cm H2O
- **Cardiovascular:** Irregularly irregular rhythm, tachycardic, S3 gallop present, 2/6 systolic murmur best heard at apex
- **Respiratory:** Bilateral crackles to mid-lung fields, decreased air entry at bases
- **Abdomen:** Soft, non-tender, mild hepatomegaly with liver edge palpable 3 cm below costal margin
- **Extremities:** 3+ pitting edema bilaterally to mid-thigh, diminished peripheral pulses
- **Skin:** Cool extremities, mild cyanosis of nail beds

Laboratory Findings on Admission

- **CBC:** WBC $12.4 \times 10^9/L$, Hgb 9.7 g/dL, Plt $278 \times 10^9/L$
- **BMP:** Na 133 mmol/L, K 5.1 mmol/L, Cl 98 mmol/L, CO2 22 mmol/L, BUN 45 mg/dL, Cr 1.87 mg/dL (baseline 1.3)
- **Cardiac Enzymes:** Initial Troponin-I 0.28 ng/mL (elevated), peaked at 3.45 ng/mL
- **BNP:** 4,560 pg/mL
- **ABG:** pH 7.33, pCO2 48 mmHg, pO2 62 mmHg on 2L NC, HCO3 24 mEq/L
- **HbA1c:** 9.4%
- **LFTs:** ALT 45 U/L, AST 52 U/L, Albumin 3.2 g/dL

- **Coagulation:** INR 1.2, PTT 32 sec
- **Other:** Ferritin 48 ng/mL, TSAT 15%, Folate and B12 within normal limits

Diagnostic Studies

- **ECG:** Atrial fibrillation with RVR (rate 110-130), non-specific ST-T wave changes in anterolateral leads
- **Chest X-ray:** Pulmonary vascular congestion, bilateral pleural effusions (R>L), cardiomegaly
- **Echocardiogram:** LVEF 52%, grade II diastolic dysfunction, mild-moderate MR, dilated LA (volume index 42 mL/m²), moderate TR with PASP 55 mmHg, RV mildly dilated with preserved function
- **Coronary Angiography (4/17/2025):** 85% stenosis of proximal LAD, 60% stenosis of mid-RCA, 40% stenosis of D1 branch. PCI with DES to proximal LAD performed with good result
- **CT Chest (4/19/2025):** Moderate bilateral pleural effusions, basilar consolidations consistent with pneumonia, diffuse ground-glass opacities consistent with pulmonary edema
- **Lower Extremity Doppler (4/18/2025):** No evidence of DVT, severe bilateral arterial insufficiency, monophasic flow in posterior tibial arteries

Hospital Course by System

Cardiovascular

1. **Heart Failure:** Patient was admitted with ADHF with preserved EF, likely precipitated by medication non-adherence and complicated by AFib with RVR and NSTEMI. Initial management included IV furosemide 80 mg BID with good diuretic response (net negative 8L over first 5 days). Transitioned to PO furosemide on day 6 with continued improvement in congestion.
2. **NSTEMI:** Diagnosed based on rising troponins and symptoms. Coronary angiography on 4/17 revealed proximal LAD stenosis requiring DES placement. Post-procedure course uncomplicated. Started on dual antiplatelet therapy.
3. **Arrhythmia:** Presented in AFib with RVR. Initially rate-controlled with IV metoprolol, then transitioned to oral metoprolol succinate with good effect. Given CHA₂DS-VASc score of 6, anticoagulation was indicated. Started on apixaban with plan for indefinite therapy. Rhythm control strategy deferred to outpatient setting given multiple acute issues.

Pulmonary

1. **COPD:** Home medications continued. No acute exacerbation during admission.
2. **Healthcare-associated Pneumonia:** Developed fever (38.4°C) and increasing oxygen requirements on day 4. CT chest confirmed basilar consolidations. Started on IV ceftriaxone and azithromycin with good clinical response. Sputum culture grew *Klebsiella pneumoniae*, sensitive to ceftriaxone. Completed 7-day course of antibiotics with resolution of infiltrates on repeat imaging.
3. **Respiratory Status:** Initial oxygen requirement of 2L NC increased to 4L during pneumonia, successfully weaned to room air by discharge.

Renal

1. **AKI on CKD:** Baseline Cr 1.3 mg/dL, peaked at 2.1 mg/dL on day 3, likely multifactorial (CHF, contrast from angiography, initial dehydration from aggressive diuresis). IV fluids avoided due to volume overload. Medications adjusted for renal function. Gradual improvement with Cr 1.5 mg/dL at discharge.
2. **Electrolyte Management:** Required careful K⁺ supplementation during diuresis. Magnesium also supplemented.

Endocrine

1. **Diabetes Management:** Home regimen of metformin and glipizide held due to AKI. Insulin glargine and aspart sliding scale initiated. Significant hyperglycemia throughout hospitalization requiring adjustment of insulin dosing. Endocrinology consulted and recommended continuation of insulin regimen at discharge with close outpatient follow-up.
2. **Thyroid Studies:** TSH 3.4 mIU/L (normal)

Hematologic

1. **Anemia:** Workup consistent with iron deficiency anemia. Received IV iron sucrose with mild improvement in Hgb from 9.7 to 10.2 g/dL. Outpatient iron therapy and hematology follow-up arranged.

Nutrition/GI

1. **Nutritional Status:** Dietary consult for CHF and diabetic diet education.
2. **Mild transaminitis:** Attributed to hepatic congestion, improved with diuresis.

Infectious Disease

1. **Healthcare-associated pneumonia:** Treated with 7-day course of ceftriaxone and azithromycin as noted above. Afebrile for >72 hours prior to discharge.

Complications During Admission

1. **Acute delirium:** Developed on day 5-7, likely multifactorial (new environment, infection, medication effects). Resolved with treatment of pneumonia and environmental modifications.
2. **Sacral stage I pressure injury:** Detected on day 8, wound care consulted, specialized mattress and repositioning protocol implemented with improvement noted prior to discharge.

Procedures

1. Coronary angiography with PCI and DES to proximal LAD (4/17/2025)
2. Therapeutic thoracentesis, right pleural effusion: 850 mL removed (4/20/2025)

Consultations

1. Cardiology
2. Nephrology
3. Endocrinology
4. Infectious Disease
5. Wound Care

DISCHARGE CONDITION

Patient discharged in stable condition. Ambulating independently with minimal dyspnea. Euvolemic on physical exam with resolved JVD and peripheral edema. Off supplemental oxygen with SpO2 95% on room air. Vital signs stable: BP 136/78, HR 78 regular, RR 16, afebrile.

DISCHARGE LABORATORY VALUES (4/29/2025)

- **CBC:** WBC $7.8 \times 10^9/L$, Hgb 10.2 g/dL, Plt $256 \times 10^9/L$
- **BMP:** Na 138 mmol/L, K 4.3 mmol/L, Cl 101 mmol/L, CO₂ 25 mmol/L, BUN 32 mg/dL, Cr 1.5 mg/dL
- **LFTs:** Within normal limits
- **BNP:** 980 pg/mL (improved from admission)

DISCHARGE MEDICATIONS

1. **Apixaban (Eliquis)** 5 mg PO BID (reduced dose due to age > 80, weight < 60 kg, or Cr > 1.5 would be 2.5 mg BID)
2. **Aspirin** 81 mg PO daily (for 12 months post-PCI)
3. **Clopidogrel** 75 mg PO daily (for 12 months post-PCI)
4. **Metoprolol succinate (Toprol XL)** 50 mg PO daily
5. **Lisinopril** 10 mg PO daily
6. **Furosemide** 40 mg PO BID
7. **Spirolactone** 25 mg PO daily
8. **Atorvastatin** 80 mg PO daily
9. **Insulin glargine (Lantus)** 18 units subcutaneous daily at bedtime
10. **Insulin aspart (Novolog)** 4 units subcutaneous with meals and correction scale
11. **Tiotropium (Spiriva)** 18 mcg inhaled daily
12. **Budesonide/formoterol (Symbicort)** 160/4.5 mcg 2 puffs inhaled BID
13. **Albuterol/ipratropium (Combivent)** 2 puffs inhaled q6h PRN shortness of breath
14. **Ferrous sulfate** 325 mg PO TID
15. **Famotidine** 20 mg PO daily
16. **Acetaminophen** 650 mg PO q6h PRN pain/fever

MEDICATIONS DISCONTINUED DURING HOSPITALIZATION

1. Metformin (will be reassessed at follow-up after renal function stabilizes)
2. Glipizide (replaced with insulin regimen)
3. Amlodipine (blood pressure well-controlled on current regimen)

ALLERGIES

1. **Penicillin** - Rash
2. **Sulfa drugs** - Anaphylaxis

DISCHARGE INSTRUCTIONS

Activity

- Gradually increase physical activity as tolerated
- Daily walking, starting with 5-10 minutes and increasing by 5 minutes every 3 days as tolerated
- No driving for 1 week post-PCI
- No heavy lifting (>10 pounds) for 2 weeks
- May shower, avoid baths until incision site fully healed

Diet

- Low sodium (< 2g/day) heart-healthy diet
- Diabetic diet with carbohydrate counting
- Fluid restriction to 1.5 L/day total
- Avoid alcohol

Daily Weights

- Weigh yourself every morning after urinating, before breakfast, wearing similar clothing
- Record weights in log provided
- Call physician if weight increases by > 2 kg (4.4 lbs) in 3 days

Wound Care

- Keep cardiac catheterization site clean and dry
- Monitor for redness, swelling, drainage, or increased pain
- Apply silicone foam dressing to sacral area every 3 days as instructed by wound care nurse

Monitoring

- Check blood glucose 4 times daily (before meals and at bedtime), record in log
- Check blood pressure daily and record
- Call physician if systolic BP < 100 or > 160, or if diastolic BP > 100

Special Instructions

1. **Medication Adherence:** Take all medications as prescribed. DO NOT discontinue any medications without consulting your healthcare provider.
2. **Diet:** Follow-up with dietitian scheduled for detailed education on sodium restriction and diabetic diet.
3. **Symptoms:** Call provider or seek emergency care if experiencing:
 - Chest pain or pressure
 - Shortness of breath at rest or with minimal activity
 - Dizziness or syncope
 - Palpitations
 - Swelling in ankles/legs that worsens
 - Weight gain > 2 kg in 3 days
 - Blood glucose > 300 mg/dL despite correction dose
 - Signs of bleeding (unusual bruising, blood in urine/stool, severe headache)

FOLLOW-UP APPOINTMENTS

1. **Primary Care (Dr. Marco Verdi):** 1 week post-discharge (05/06/2025) at 10:00 AM
2. **Cardiology (Dr. Elena Bianchi):** 2 weeks post-discharge (05/13/2025) at 2:00 PM
3. **Cardiology Nurse (Heart Failure Clinic):** 1 week post-discharge (05/06/2025) at 1:00 PM
4. **Endocrinology (Dr. Sofia Neri):** 2 weeks post-discharge (05/13/2025) at 11:00 AM
5. **Nephrology (Dr. Giuseppe Blu):** 3 weeks post-discharge (05/20/2025) at 9:00 AM
6. **Hematology (Dr. Anna Gialli):** 4 weeks post-discharge (05/27/2025) at 3:00 PM
7. **Cardiac Rehabilitation:** Initial evaluation scheduled for (05/15/2025) at 10:00 AM

PREVENTIVE CARE NEEDS

- Influenza vaccine annually (received 10/2024)
- Pneumococcal vaccine: PPSV23 administered on 03/2023, PCV20 due 03/2028
- Shingles vaccine: Series completed 08/2022
- COVID-19 vaccination: Last booster 09/2024
- Diabetic eye exam: Last completed 02/2025, next due 02/2026
- Diabetic foot exam: Completed during hospitalization, next due in 3 months
- Colonoscopy: Last completed 01/2023, next due 01/2033
- Mammogram: Last completed 10/2024, next due 10/2025

DISCHARGE PLANNING

Patient discharged to home with the following services:

1. Home health nursing: 2x weekly for 2 weeks for medication management, vital sign monitoring, and wound assessment
2. Physical therapy: 2x weekly for 3 weeks for progressive exercise program and mobility assessment
3. Home health aide: 3x weekly for 2 weeks for assistance with ADLs

Equipment provided:

1. Digital scale for daily weights
2. Glucometer and testing supplies
3. Blood pressure monitor
4. Walker for stability as needed

Dictated by: Dr. Elena Bianchi

Dictation Date: 04/29/2025

Electronically signed by: Dr. Elena Bianchi

Signature Date: 04/29/2025